SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Gemcitabine and cisplatin · lung cancer

Advanced non-small-cell lung cancer · standalone chemotherapy · NCCP 00281 · version 7 · 21-day cycle

Patient name	DOB	Hospital no.	Date

Treatment context
These two chemotherapy medicines aim to slow or shrink advanced non-small-cell lung cancer and help control symptoms. Benefit is not guaranteed. Your team explains the aim for your own situation.

Schedule
A cycle lasts 21 days (3 weeks). Day 1: gemcitabine and cisplatin. Day 8: gemcitabine alone after separate blood tests and review. Days 9–21: no scheduled chemotherapy. The usual planned course is 4–6 cycles, with earlier changes for progression or unacceptable effects.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Gemcitabine
CHEMOTHERAPY

- ☐ Low blood counts and infection
- ☐ Nausea, fatigue and flu-like symptoms
- ☐ Skin rash and swelling
- ☐ Rare serious lung or kidney injury

Cisplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts, infection and bleeding
- ☐ Nausea, vomiting and fatigue
- ☐ Mouth soreness and diarrhoea
- ☐ Kidney problems
- ☐ Allergic or infusion reactions
- ☐ Hearing changes or tinnitus, which can persist
- ☐ Numbness or tingling, which can persist
- ☐ Electrolyte changes and need for hydration

Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan. Cisplatin needs prescribed IV hydration, electrolyte replacement and kidney, hearing and nerve checks. Follow your own fluid advice; tell the team if you have heart failure or a fluid restriction. Discuss potentially serious or fatal infection, organ injury, fertility effects and lasting nerve/hearing changes, as applicable. This guide covers standalone chemotherapy for advanced NSCLC. If nivolumab is part of your treatment before surgery, use the separate 00849 guide for gemcitabine/cisplatin; that is a different three-cycle plan.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00281 v7.

SACTCheck v0.84.28
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